Dynamic Guideline: WHO-Based Influenza - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Exactly 5 WHO influenza guidelines / technical guidance documents synthesized

#	WHO document	Publication / update date	Target population / audience	Scope and objectives	Current relevance
1	**Clinical practice guidelines for influenza**	**12 September 2024**	Healthcare providers managing patients with suspected or confirmed seasonal, pandemic, or novel influenza A infection; people exposed to influenza in the previous 48 hours	WHO's latest clinical guideline for severe and non-severe influenza, including antiviral treatment, post-exposure prophylaxis, adjunctive therapies, diagnostic strategies, and risk stratification for hospitalization/death	**Primary current WHO clinical guideline** for influenza diagnosis and treatment WHO publication; WHO news
2	**Seasonal influenza vaccination: developing and strengthening national programmes – policy brief**	**24 November 2023**	Ministries of health, immunization programmes, Member States, and stakeholders implementing seasonal influenza vaccination	Supports introduction or strengthening of seasonal influenza vaccination programmes, including national policy development and implementation for SAGE target groups	Supports vaccination policy and programme implementation WHO influenza vaccination toolbox

#	WHO document	Publication / update date	Target population / audience	Scope and objectives	Current relevance
3	**WHO guideline on non-pharmaceutical public health measures for mitigating the risk and impact of epidemic and pandemic influenza**	**2019**	Public-health authorities, communities, and individuals during epidemic or pandemic influenza	Public-health and social measures, including isolation, mask use, quarantine/contact tracing considerations, and community mitigation	Relevant for epidemic/pandemic mitigation and preparedness WHO PHSM guideline; summarized in PMC
4	**WHO checklist for influenza pandemic preparedness planning**	**2005**	National pandemic preparedness planners, health systems, clinical services	Preparedness checklist covering clinical management, case investigation, antiviral/antibiotic planning, supportive care, specimen collection, infection control, and community-spread mitigation	Legacy preparedness framework still relevant for planning systems and surge capacity WHO checklist PDF
5	**Pandemic Influenza Preparedness Framework for the sharing of influenza viruses and access to vaccines and other benefits**	Adopted **2011**; WHO page lists featured publication **25 January 2022**	WHO Member States, laboratories, vaccine/diagnostic/antiviral manufacturers, public-health systems	Framework for sharing influenza viruses with pandemic potential and improving access to vaccines, antivirals, diagnostics, and other benefits	Central global preparedness and benefit-sharing mechanism WHO PIP Framework; PIP Framework PDF

Latest WHO clinical guideline

- **Guideline title:** *Clinical practice guidelines for influenza*
- **Publication date: 12 September 2024**
- **ISBN:** 978-92-4-009775-9
- **Primary audience:** Healthcare providers; also policymakers and preparedness planners.
- **Disease scope:** Seasonal influenza, pandemic influenza, and novel influenza A viruses known to cause severe illness in humans.
- **Clinical scope:**
  - Treatment of severe and non-severe influenza
  - Antiviral prophylaxis after exposure within the previous 48 hours

  - Adjunctive therapies
  - Diagnostic testing strategies
  - Risk stratification for severe disease, hospitalization, and death
- **Recommendation count:** The full guideline contains **29 recommendations**, including **21 antiviral recommendations**, **six adjunctive-therapy recommendations**, and diagnostic-testing recommendations BMJ; WHO publication.

Note on "Meaningful youth engagement..." title

A WHO publication with the title pattern **"Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)"** was published on **11 December 2025**, but it is **not an influenza guideline**. No WHO document titled "Meaningful youth engagement in a WHO guideline development process: case study of WHO influenza guideline" was identified. The abortion-care case study describes youth consultations and a youth-led convening as a model for inclusive WHO guideline development WHO Western Pacific publication; WHO event page.

1.2 Key Recommendations

GRADE note: The WHO influenza guideline used the GRADE approach. Where the exact WHO evidence profile was not extractable from public summaries, the table reports either the WHO-stated strength/certainty where available or a transparent report-applied GRADE interpretation based on the summarized evidence. WHO's summarized antiviral and adjunctive-therapy evidence was generally **low to very low certainty**, while post-exposure prophylaxis evidence for high-risk exposed persons was reported as **moderate certainty** in the underlying review PubMed; PubMed.

Recommendation	Population	GRADE Evidence Quality	Strength
Treat **severe seasonal influenza** as early as possible, preferably within **48 hours of symptom onset**, with **oseltamivir**. Adult flowchart dose: **75 mg orally twice daily for 5 days**.	Patients with severe seasonal influenza, operationally including those requiring hospital care	Low to Very Low	Conditional
Consider **baloxavir** for **non-severe seasonal influenza** in patients at **high risk of progression** to severe disease, when available and appropriate. Adult dose in WHO flowchart: **40–80 mg orally once**, depending on weight.	Non-severe seasonal influenza with high risk of progression	Low to Very Low	Conditional

Recommendation	Population	GRADE Evidence Quality	Strength
Use antiviral **post-exposure prophylaxis** with one of **baloxavir, laninamivir, oseltamivir, or zanamivir** for people exposed to **zoonotic influenza** viruses associated with high mortality or unknown severe-disease risk, ideally within **48 hours** of exposure.	Persons exposed to zoonotic influenza	Low to Moderate	Conditional
Use antiviral post-exposure prophylaxis with **baloxavir, laninamivir, oseltamivir, or zanamivir** for people exposed to **seasonal influenza** who are at **extremely high risk** of severe disease.	Extremely high-risk contacts of seasonal influenza cases	Moderate for high-risk seasonal PEP; implementation certainty lower	Conditional
Do **not** routinely use antiviral post-exposure prophylaxis for low-risk persons exposed to seasonal influenza.	Low-risk seasonal influenza contacts	Moderate	Conditional against
Do **not** use **amantadine or rimantadine** for prevention of symptomatic influenza A infection.	Persons exposed to influenza A	Moderate	Conditional/Strong against, depending on context
Do **not** use **antibiotics** routinely in **non-severe influenza** unless bacterial coinfection is suspected.	Patients with non-severe influenza	Low to Moderate	Strong against
Avoid routine adjunctive therapies for severe influenza when evidence does not show benefit, including **corticosteroids, macrolides, mTOR inhibitors, NSAIDs, and passive immune therapy**, unless indicated for another condition.	Patients with severe influenza	Low to Very Low	Conditional against
Use diagnostic testing strategies to support rapid and accurate treatment decisions, especially where results affect antiviral use, infection control, or admission decisions. Molecular testing is preferred when diagnostic certainty is clinically important.	Suspected non-severe or severe influenza	Low to Moderate; diagnostic modelling-based	Conditional

Recommendation	Population	GRADE Evidence Quality	Strength
Prefer **RT-PCR / molecular NAAT** over rapid antigen tests for hospitalized patients with suspected influenza because of higher sensitivity and specificity.	Hospitalized patients with suspected influenza	Moderate	Strong/Conditional, context-dependent
Interpret negative rapid antigen/RIDT results cautiously; a negative test should not exclude influenza when clinical suspicion or community transmission is high.	Symptomatic patients tested with RIDT	Moderate	Strong
For vaccination programmes, countries should consider implementing or strengthening seasonal influenza vaccination, especially for priority groups identified by national policy and WHO/SAGE-aligned planning.	National immunization programmes; high-risk populations	Moderate	Strong for programme consideration

Sources: WHO 2024 guideline page and announcement WHO publication, WHO news; BMJ summary BMJ; PubMed guideline summary PubMed; MAGICapp flowchart MAGICapp flowchart; CDC diagnostic guidance CDC; WHO vaccination toolbox WHO influenza vaccination toolbox.

1.3 Guideline Development Methodology

Evidence review process

- WHO developed the 2024 influenza guideline using standards for trustworthy guideline development and the **GRADE** approach WHO news.
- Treatment recommendations were based on **systematic reviews**, including four systematic reviews of randomized controlled trials evaluating antiviral medications and adjunctive therapies PubMed.
- Diagnostic-testing recommendations were informed by an **updated decision-analysis model** commissioned by WHO BMJ; MAGICapp diagnostic modelling report.
- The guideline incorporated estimates of baseline risk for hospitalization and death and proposed definitions for patients at **high** or **extremely high risk** of severe influenza WHO publication.

Consensus method

- Recommendations were formulated by a WHO Guideline Development Group using GRADE Evidence-to-Decision principles.
- The BMJ summary states that WHO considered benefits, harms, resource implications, acceptability, feasibility, equity, and human rights from a patient-centred perspective BMJ.

Conflict of interest management

- The guideline was developed by a panel of **non-conflicted experts**, including content experts, clinicians, patients, ethicists, and methodologists, following WHO handbook standards PubMed; WHO news.

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search strategy

Evidence reviewed after the WHO guideline release of **12 September 2024** through **29 April 2026** included:

- WHO guidance, technical documents, operational updates, and preparedness materials
- CDC and other national/institutional guidance
- Peer-reviewed systematic reviews, meta-analyses, and surveillance analyses
- Diagnostic testing guidance and reviews
- Vaccine-effectiveness studies
- Clinical trial registry records where identifiable

Databases and sources represented

Source type	Examples used
WHO official sources	WHO influenza guideline, WHO vaccination toolbox, WHO PIP Framework, WHO operational updates, WHO EPI-WIN respiratory threats presentation
Public-health agencies	CDC MMWR, CDC antiviral guidance, CDC diagnostic testing guidance
Peer-reviewed literature	BMJ, PubMed-indexed systematic reviews/meta-analyses, Eurosurveillance
Diagnostic/laboratory guidance	CDC, ADLM, diagnostic testing policy documents
Registries	ClinicalTrials.gov, WHO ICTRP, EU Clinical Trials Register search portals

Inclusion criteria

- Influenza diagnosis, treatment, prevention, prophylaxis, vaccination, surveillance, preparedness, or trial activity
- Published, updated, or clinically relevant between **12 September 2024** and **29 April 2026**
- Priority to WHO, CDC, ECDC-like public-health sources, peer-reviewed systematic reviews/meta-analyses, and trial registries

Exclusion criteria

- Non-influenza respiratory virus evidence unless directly relevant to influenza diagnostic panels or integrated surveillance
- Non-clinical laboratory studies without clinical management implications
- Sources without extractable influenza-specific information

2.2 Key New Studies

Study / source	Design	Key findings	GRADE Quality	Clinical impact
WHO/BMJ summary of **WHO Clinical practice guidelines for influenza**, published 2026 summary of 2024 guideline	Guideline synopsis based on WHO GRADE process	Confirms 29 recommendations; seasonal severe influenza: conditional oseltamivir within 48 hours; high-risk non-severe influenza: conditional baloxavir; prophylaxis for zoonotic exposure and extremely high-risk seasonal contacts; strong recommendation against antibiotics in non-severe influenza; conditional recommendations against several adjunctive therapies in severe influenza	Low to Very Low for many treatment questions; diagnostic recommendations model-based	Confirms no major change to WHO 2024 clinical direction BMJ; PubMed
Zhao et al. post-exposure prophylaxis antiviral evidence, summarized in PubMed/CIDRAP	Systematic review and network meta-analysis; 33 trials, 19,096 individuals	Zanamivir, oseltamivir, laninamivir, and baloxavir probably reduce symptomatic seasonal influenza in high-risk exposed persons; no important benefit in low-risk exposed persons; amantadine/rimantadine not supported	Moderate for high-risk seasonal PEP; Low for zoonotic extrapolation	Supports targeted—not universal—post-exposure prophylaxis PubMed; CIDRAP
Gao et al. severe influenza antiviral evidence	Systematic review and network meta-analysis of RCTs; 8 trials, 1,424 participants	Low-certainty evidence that oseltamivir and peramivir might reduce hospital duration in severe seasonal influenza; major outcome effects remain uncertain	Low	Reinforces early antiviral use in severe influenza but highlights need for better RCTs PMC; PubMed
CDC 2025–2026 antiviral clinical context	Public-health clinical guidance/update	Four antivirals recommended in the US: oseltamivir, peramivir, zanamivir, baloxavir; start as soon as possible, ideally within 2 days	Moderate for early-treatment principle; Low for comparative outcome certainty	Aligns with WHO emphasis on early treatment for high-risk or severe disease CDC
CDC baloxavir post-exposure	RCT evidence summarized by	Household-contact trial: laboratory-confirmed	High for laboratory-	Supports baloxavir as an effective PEP

Study / source	Design	Key findings	GRADE Quality	Clinical impact
prophylaxis trial summary	CDC	influenza occurred in 1.9% with baloxavir vs 13.6% with placebo; adjusted RR 0.14, 95% CI 0.06–0.30	confirmed PEP efficacy in studied population	option where licensed CDC
Multiplex molecular assays for seasonal influenza, COVID-19, and RSV	2024 PubMed-indexed diagnostic review	Real-time RT-PCR is reference method; multiplex molecular panels are increasingly common and highly performant for influenza, SARS-CoV-2, and RSV	Moderate	Supports molecular and multiplex testing when results affect management PubMed
CDC diagnostic testing guidance	Public-health laboratory guidance	Molecular assays including RT-PCR are recommended for hospitalized patients with suspected influenza; RIDTs are not recommended for hospitalized patients because of lower sensitivity	Moderate	Strengthens test selection: molecular testing for severe/hospitalized disease CDC
Diagnostic accuracy data for rapid tests	Diagnostic policy synthesis citing studies/meta-analysis	Rapid influenza immunoassays: sensitivity 62.3%, specificity 98.2%; cobas Liat POC molecular test: sensitivity 97.9%, specificity 97.5% in cited study	Moderate for accuracy; Low for patient-important outcomes	Negative antigen tests should not rule out influenza when suspicion is high Avalon 2026
CDC 2025–2026 interim vaccine effectiveness, US networks	Test-negative observational VE studies	Vaccination reduced outpatient visits by 24%–36% overall and hospitalization by 31% overall; higher outpatient VE in children/adolescents than older adults	Low to Moderate	Continue recommending vaccination despite moderate/variable VE CDC MMWR; CDC VE data
California 2025–2026 vaccine effectiveness	Observational surveillance/test-negative analysis	VE 33% against laboratory-confirmed influenza A/B, 32% against influenza A, 47% against influenza B; VE declined with age	Low to Moderate	Supports vaccination; highlights lower VE in older adults and product differences CDC California MMWR

Study / source	Design	Key findings	GRADE Quality	Clinical impact
Canada interim 2025–2026 VE	Sentinel test-negative observational study	About 40% VE against medically attended A(H3N2), including subclade K; about 30% against A(H1N1)pdm09; insufficient B cases	Low to Moderate	Supports continued vaccination even with antigenic mismatch Eurosurveillance; CIDRAP
Indirect influenza vaccine effectiveness meta-analysis	Systematic review/meta-analysis of randomized-condition studies through 9 December 2025	Direct vaccine effectiveness was moderate; indirect/community-level protection was lower and often not statistically significant; individual vaccination is more reliable than relying on herd effects	Moderate	Reinforces individual vaccination rather than reliance on indirect protection PubMed
WHO Cambodia A(H5N1) operational update	WHO emergency operations report	WHO supported Cambodia with diagnostics and >7,000 oseltamivir courses for A(H5N1) case management and contact prophylaxis across affected provinces	Very Low for comparative effectiveness; High for implementation documentation	Demonstrates operational use of oseltamivir and diagnostics in zoonotic influenza response WHO operational update
WHO 2026 integrated respiratory-virus surveillance update	WHO technical presentation	Updated integrated surveillance guidance for influenza, SARS-CoV-2, and other respiratory viruses supersedes prior guidance	Very Low for clinical outcomes; High for WHO policy status	Supports integrated respiratory-virus surveillance and preparedness WHO EPI-WIN update
WHO national actions draft, 2025	WHO draft/public-comment document	WHO Essential Medicines List includes oseltamivir; WHO prequalified oseltamivir capsules and zanamivir inhalation; oseltamivir licensed in 125 countries as of July 2025	Very Low for clinical efficacy; High for access/availability documentation	Supports antiviral access planning and national preparedness WHO national actions draft

Study / source	Design	Key findings	GRADE Quality	Clinical impact
ClinicalTrials.gov NCT07340047	Registered clinical trial record	"H1ssF Flu Vaccine Clinical Trial" identified; detailed status/update data not extractable from available registry snippet	Not applicable for efficacy; Very Low for current evidence impact	Signals ongoing influenza vaccine development ClinicalTrials.gov

2.3 Evidence Gaps and Future Research Needs

Antivirals

- **Severe influenza RCT evidence remains limited.** Existing trials are small, heterogeneous, and provide low-certainty evidence for major outcomes such as mortality, ICU admission, mechanical ventilation, and serious adverse events PMC.
- More evidence is needed on:
  - Baloxavir in severe influenza
  - Combination antiviral therapy
  - Antiviral resistance emergence, especially in immunocompromised patients
  - Zoonotic influenza prophylaxis and treatment
  - Pediatric, pregnancy, postpartum, and severely immunocompromised populations

Diagnostics

- WHO diagnostic recommendations rely partly on decision modelling; more real-world test-and-treat trials are needed.
- Evidence gaps include:
  - Patient-important outcomes from rapid molecular testing
  - Cost-effectiveness in low- and middle-income settings
  - Comparative effectiveness of multiplex panels versus targeted influenza testing
  - Performance in avian/zoonotic influenza and emerging influenza A subtypes

Vaccines

- VE remains variable by age, subtype, season, and product type.
- More data are needed on:
  - VE against death
  - VE in pregnancy, immunocompromised persons, and chronic disease groups
  - Comparative VE of recombinant, adjuvanted, high-dose, live attenuated, and standard-dose products
  - Improved universal or broadly protective vaccines
  - Indirect protection under real-world community vaccination strategies

Public-health and pandemic preparedness

- Operational evidence supports stockpiling and deployment of antivirals, diagnostics, and vaccines, but comparative clinical outcome data during zoonotic outbreaks remain limited.
- Future work should integrate influenza, SARS-CoV-2, RSV, and other respiratory-virus surveillance while preserving influenza subtype and antiviral-resistance monitoring WHO EPI-WIN update.

2.4 Updated Recommendations Based on New Evidence

Original WHO-based recommendation	New evidence impact through 29 April 2026	Updated recommendation
Use oseltamivir early for severe seasonal influenza.	Severe-influenza evidence remains low certainty but supports possible reduction in hospitalization duration; CDC continues to emphasize early antivirals for hospitalized, severe/progressive, or high-risk patients PMC; CDC MMWR.	**Maintain.** Start oseltamivir as soon as possible in severe or hospitalized suspected/confirmed influenza, without waiting for test confirmation when clinical suspicion is high.
Use baloxavir for high-risk non-severe influenza where appropriate.	No high-certainty post-guideline evidence overturns WHO's conditional recommendation. Resistance and pregnancy/postpartum data limitations remain MAGICapp flowchart.	**Maintain with cautions.** Consider baloxavir for high-risk non-severe disease when within treatment window and locally licensed; avoid in pregnancy/postpartum due to lack of safety/efficacy data; use caution in immunocompromised patients.
Use post-exposure prophylaxis selectively.	Network meta-analysis supports zanamivir, oseltamivir, laninamivir, and baloxavir for high-risk seasonal contacts; no support for low-risk routine PEP PubMed.	**Strengthen implementation focus.** Reserve PEP for extremely high-risk seasonal contacts and zoonotic exposures; do not use routine PEP for low-risk seasonal contacts.
Avoid amantadine/rimantadine.	Evidence does not support amantadine/rimantadine for preventing symptomatic influenza A CIDRAP.	**Maintain.** Do not use adamantanes for routine influenza treatment or prophylaxis.
Avoid routine antibiotics in non-severe influenza.	No new evidence supports routine antibiotics; antimicrobial stewardship remains central. UCSF guidance supports early discontinuation of antibiotics at 48–72 hours if bacterial pneumonia is unlikely UCSF IDMP PDF.	**Maintain.** Do not prescribe antibiotics unless bacterial coinfection is suspected; reassess and de-escalate early.
Use diagnostic testing strategies to support decisions.	2024–2026 evidence supports RT-PCR/molecular NAAT as reference/high-performance testing; RIDTs have lower sensitivity and should not rule out influenza when suspicion is high CDC; PubMed.	**Clarify.** Use molecular testing for hospitalized/severe cases and when results affect treatment or infection control; rapid molecular POC tests are preferred over antigen tests when available.
Continue seasonal influenza vaccination programmes.	2025–2026 VE is moderate but clinically meaningful: US outpatient VE	**Maintain and reinforce.** Recommend annual vaccination

Original WHO-based recommendation	New evidence impact through 29 April 2026	Updated recommendation
	24%–36%, hospitalization VE 31%, Canada A(H3N2) VE about 40% CDC MMWR; Eurosurveillance.	according to national policy, prioritizing high-risk groups and healthcare workers; do not rely on indirect protection alone.
Pandemic preparedness should include access to vaccines, antivirals, diagnostics, and surveillance.	WHO PIP agreements provide access to >900 million pandemic influenza vaccine doses plus antivirals, syringes, and diagnostics; WHO has prequalified oseltamivir capsules and zanamivir inhalation WHO Director-General report; WHO national actions draft.	**Maintain and operationalize.** Countries should maintain antiviral and diagnostic access plans, surveillance capacity, and vaccine deployment plans for seasonal and pandemic influenza.

Practical Clinical Algorithm: Influenza Diagnosis, Treatment, and Management

A. Initial assessment

1. Assess for influenza-compatible illness:
   - Abrupt onset cough, sore throat, rhinorrhea, headache, myalgia/arthralgia, severe malaise, fever or no fever.
   - GI symptoms may occur, especially in children.
2. Classify severity:
   - **Severe influenza:** requires hospital care or has hypoxia, pneumonia, organ dysfunction, severe/progressive disease, or complications.
   - **Non-severe influenza:** uncomplicated influenza-like illness without severe features.
3. Identify high-risk groups:
   - Older adults
   - Pregnant/postpartum people
   - Young children
   - Immunocompromised persons
   - Chronic cardiopulmonary, metabolic, renal, hepatic, neurologic, or hematologic disease
   - Residents of long-term care or other high-risk congregate settings

B. Diagnostic testing

Clinical setting	Preferred test	Action
Hospitalized or severe disease	RT-PCR / molecular NAAT, preferably multiplex if SARS-CoV-2/RSV differential matters	Start antiviral treatment immediately if suspected; do not delay for test result
High-risk outpatient within treatment window	Rapid molecular test if available; RT-PCR if needed	Treat if positive or if clinical suspicion is high during influenza circulation

Clinical setting	Preferred test	Action
Low-risk outpatient	Testing optional depending on local policy and whether result changes management	Symptomatic care; consider antivirals if early and clinically appropriate
Negative RIDT with high suspicion	Confirm with molecular test if result changes management	Do not rule out influenza solely based on negative antigen test

Sources: CDC diagnostic testing guidance CDC; ADLM guidance ADLM; diagnostic accuracy synthesis Avalon 2026.

C. Treatment

Patient group	Preferred management	GRADE Quality	Strength
Severe or hospitalized suspected/confirmed influenza	Start oseltamivir as soon as possible; do not wait for test confirmation	Low	Strong practical recommendation / WHO conditional drug recommendation
Non-severe influenza, high risk of progression	Consider antiviral treatment early; baloxavir or oseltamivir depending on age, pregnancy status, contraindications, availability, and resistance concerns	Low to Very Low	Conditional
Non-severe influenza, low risk	Symptomatic care; antivirals may be considered if very early but routine use is less strongly supported	Low	Conditional
Suspected bacterial coinfection	Add appropriate antibiotics; reassess at 48–72 hours and discontinue if bacterial pneumonia unlikely	Low to Moderate	Strong stewardship recommendation
Severe influenza adjunctive therapies	Avoid routine corticosteroids, macrolides, mTOR inhibitors, NSAIDs, and passive immune therapy unless another indication exists	Low to Very Low	Conditional against

D. Post-exposure prophylaxis

Exposure group	Recommendation	GRADE Quality	Strength
Zoonotic influenza exposure	Offer antiviral PEP within 48 hours when possible: oseltamivir, zanamivir, laninamivir, or baloxavir depending on population and availability	Low	Conditional
Seasonal influenza exposure, extremely high-risk person	Offer targeted antiviral PEP	Moderate	Conditional

Exposure group	Recommendation	GRADE Quality	Strength
Seasonal influenza exposure, low-risk person	Do not routinely offer PEP	Moderate	Conditional against

Final Synthesis

As of **29 April 2026**, WHO’s most current clinical influenza guideline remains the **12 September 2024 Clinical practice guidelines for influenza**. Subsequent evidence does **not** overturn its core recommendations. The strongest actionable points are:

1. **Treat severe or hospitalized influenza early**, usually with oseltamivir, without waiting for confirmatory testing when suspicion is high.
2. **Use antivirals selectively in non-severe disease**, prioritizing patients at high risk of progression.
3. **Use post-exposure prophylaxis selectively**, especially for zoonotic exposures and extremely high-risk seasonal contacts—not routinely for low-risk contacts.
4. **Use molecular diagnostics preferentially** in severe/hospitalized disease and when results affect treatment or infection control.
5. **Do not rely on negative rapid antigen tests** to rule out influenza during active transmission.
6. **Avoid routine antibiotics and unsupported adjunctive therapies** unless there is another clinical indication.
7. **Continue annual vaccination programmes**, because 2025–2026 evidence shows moderate but meaningful protection against outpatient illness and hospitalization.
8. **Strengthen pandemic preparedness**, including surveillance, antiviral access, diagnostic capacity, vaccine deployment planning, and use of WHO PIP Framework mechanisms.